

D6 — Nephrology Consult

Dr. P. Gupta, Nephrology | Nephrology | 02/19/2026 16:00

Patient: Pemberton, Harold J.**MRN:** 847-29-3156**DOB:** 04/12/1951**Allergies:** Sulfa drugs (rash)

NEPHROLOGY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/19/2026 16:00 | Provider: Dr. P. Gupta, Nephrology
Reason for Consult: CKD management, electrolyte optimization

ASSESSMENT:
Mr. Pemberton with CKD Stage 3b (baseline Cr 1.5, eGFR ~38). Currently Cr 1.5, at baseline. Presenting with COPD exacerbation, now with concern for CHF exacerbation. If diuresis is initiated, close electrolyte monitoring will be essential given CKD and multiple medications affecting potassium homeostasis.

- RECOMMENDATIONS:
1. Hold metformin until acute illness resolves and clinical picture stabilizes. Contraindicated if AKI develops.
 2. Allopurinol 100 mg daily – dose appropriate for current eGFR. Continue.
 3. Monitor BMP daily, especially K+ and Cr, particularly if diuresis is initiated.
 4. Spironolactone: acceptable to continue at current dose but monitor K+ closely with any renal changes.
 5. ACEi (lisinopril): continue but hold if Cr rises > 30% from baseline.

We will follow along during this admission.

Electronically signed by: Dr. P. Gupta, MD
Date: 02/19/2026 16:30